Sunrise Multispeciality Hospital

Sample document for the doctriage demo. All names and details are fictional.

DISCHARGE SUMMARY

Patient: Ramesh Iyer **Age/Sex:** 41 / Male **IP No.:** SMH-IP-26-30418
Admitted: 12 June 2026, 19:10 hrs **Discharged:** 15 June 2026, 11:00 hrs
Consultant: Dr. Kavita Rao, MS (Orthopaedics) **Ward:** General Surgical, Bed 14
Insurance: Policy POL-2024-00483 (motor, personal accident cover) **Police reference:** FIR No. 2026/1142

History of present illness

The patient was the driver of a car that was hit from behind by another vehicle while stopped at a traffic signal on the Western Express Highway at about 17:45 hrs on 12 June 2026. He was wearing a seat belt and the airbags did not deploy. He did not lose consciousness. He walked out of the car himself, but about an hour later he developed pain and stiffness in the neck, a headache at the back of the head, and tingling in the right hand. He was brought to the emergency department by a family member. There is no history of vomiting, fits, or bleeding from the ears or nose. He has no previous neck problems and no other illnesses, and takes no regular medicines. He is not allergic to any known drug.

Examination on admission

Conscious, alert and oriented. Pulse 88 per minute, blood pressure 132/84 mmHg, oxygen saturation 98 percent on room air. Tenderness over the back of the neck and the muscles of both shoulders. Neck movements were painful and restricted, especially turning to the right. Power in both arms was normal. There was mildly reduced sensation over the right thumb and index finger. There were no other injuries to the chest, abdomen, or legs. A small bruise from the seat belt was seen across the left side of the chest.

Investigations

X-ray of the cervical spine (three views): normal alignment, no fracture. CT scan of the head: normal. MRI of the cervical spine on 13 June 2026: soft tissue strain of the neck muscles and ligaments; no disc injury and no pressure on the spinal cord or nerves. Blood tests including blood count, sugar and kidney function were within normal limits.

Diagnosis

Whiplash injury of the neck, Grade II, following a rear-end road traffic accident. Seat belt bruise of the chest wall.

Treatment given

He was kept under observation with a soft cervical collar, pain relief, and ice packs. Physiotherapy was started on day two with gentle range-of-motion exercises. The tingling in the right hand settled by the third day. At discharge he was walking comfortably, the neck pain had reduced from 7 out of 10 to 3 out of 10, and neck movement had improved.

Medicines on discharge

1. Tab. Aceclofenac 100 mg, twice a day after food, for 5 days.
2. Tab. Pantoprazole 40 mg, once a day before breakfast, for 5 days.

3. Tab. Thiocolchicoside 4 mg, twice a day, for 5 days.
4. Diclofenac gel, apply on the neck three times a day.

Advice on discharge

Use the soft collar for one more week, only while travelling. Continue physiotherapy, ten sessions over the next three weeks. Avoid driving for two weeks and avoid lifting heavy weights for four weeks. He may return to desk work after two weeks. Return to the emergency department at once if there is weakness in the arms, worsening numbness, or severe headache. Review in the orthopaedic clinic on 22 June 2026.

Billing summary

Room charges (3 days): INR 13,500. Doctor and nursing charges: INR 9,000. X-ray, CT and MRI: INR 14,800. Medicines and consumables: INR 4,350. Physiotherapy in hospital: INR 2,400. Total bill: INR 44,050. Amount paid by the patient at discharge: INR 10,000. Balance INR 34,050 submitted to the insurer for cashless settlement.

Dr. Kavita Rao, Consultant Orthopaedic Surgeon